

University Cancer Institute - Hematology & BMT Survivorship Clinic
SURVIVORSHIP CARE PLAN

PATIENT: RAMIREZ, Louis (MRN AML073)

DOB: September 2, 1969

DATE OF PLAN GENERATION: March 28, 2024

Dear Mr. Ramirez,

Congratulations on reaching over two years since your diagnosis of Acute Myeloid Leukemia (AML). You have successfully completed a rigorous course of treatment and remain in a durable remission. This document serves as a comprehensive summary of your medical journey and a guide for your future health care. It is a tool for you, your primary care physician, and any other specialists involved in your care.

1. DIAGNOSTIC SUMMARY

- **Cancer Diagnosis:** Acute Myeloid Leukemia (AML)
- **Date of Diagnosis:** February 19, 2022
- **Presenting Symptoms:** You initially presented to your primary care physician with several weeks of worsening fatigue, shortness of breath during your usual runs, and unexplained bruising on your arms and legs.
- **Key Diagnostic Features:**
 - A bone marrow biopsy confirmed the diagnosis with 68% myeloblasts.
 - Molecular testing was critical: it showed a mutation in the **NPM1 gene** but was **negative** for any FLT3 mutations.
 - Cytogenetic analysis (karyotype) was normal.
 - This combination placed you in the **ELN 2022 Favorable-risk** category, which is associated with a very good prognosis with standard chemotherapy.

2. TREATMENT SUMMARY

Your treatment was administered with curative intent.

- **Treatment Regimen:** You were enrolled in a clinical trial and received induction and consolidation therapy with **CPX-351 (Vyxeos)**. This is a liposomal formulation containing a fixed-ratio combination of Daunorubicin and Cytarabine.
- **Induction Phase (February 27, 2022):** You received one cycle of induction therapy over a 5-day period as an inpatient. This initial phase was complicated by a prolonged period of profound myelosuppression and a documented episode of neutropenic fever requiring intravenous antibiotics, but you responded well. A bone marrow biopsy at the end of March 2022 confirmed you had achieved a **Complete Remission (CR)**.
- **Consolidation Phase (April 2022 - July 2022):** You received two additional cycles of consolidation therapy with CPX-351 to eliminate any residual leukemia cells.
- **End of Active Therapy:** Your final dose of chemotherapy was in July 2022. You have now been in continuous remission for **25 months**.

3. ONGOING FOLLOW-UP & SURVEILLANCE

Your risk of relapse is now very low, but not zero. We will continue to monitor you, but with decreasing intensity over time.

Test/Appointment	Current Frequency	Plan Moving Forward
Hematology Clinic Visit	Every 4 months	Will transition to every 6 months for the next two years, then annually until year 5 post-treatment.
Complete Blood Count (CBC)	Every 4 months (at visits)	Will transition to every 6 months with visits.
NPM1 MRD Monitoring	Every 6 months (via peripheral blood PCR)	Will continue annually for the next 3 years.
Bone Marrow Biopsy	No longer routinely scheduled	Only if there is a concerning change in your blood counts or new symptoms.

4. POTENTIAL LONG-TERM & LATE EFFECTS OF YOUR TREATMENT

The Daunorubicin component of CPX-351 is an anthracycline, which carries a lifetime cumulative dose limit due to the risk of cardiotoxicity.

- **Potential Issue:** Weakening of the heart muscle (cardiomyopathy), which can lead to congestive heart failure. The risk is low but lifelong.
- **Symptoms to Report:** New or worsening shortness of breath (especially when lying down), swelling in your ankles or feet, palpitations, or decreased exercise tolerance.
- **Screening Recommendation:** We recommend you have a baseline **echocardiogram** now, and then one every 3-5 years for long-term monitoring. It is essential that you inform all future healthcare providers that you have received anthracycline chemotherapy.

5. WELLNESS & HEALTH MAINTENANCE PLAN

- **Vaccinations:** It is critical that you get revaccinated. Your immune system was effectively reset by chemotherapy. You should receive all age-appropriate vaccines (including Tdap, MMR, Varicella, Polio, Pneumonia, and annual Influenza) as recommended by your primary care physician. **Avoid all live virus vaccines.**
- **Secondary Cancers:** While your risk is small, chemotherapy can slightly increase the lifetime risk of developing other cancers. Diligent adherence to routine health screenings is vital (e.g., colonoscopy as recommended for your age, annual skin checks).
- **Lifestyle:** Maintaining a healthy body weight, engaging in regular cardiovascular exercise, and eating a balanced diet are among the most important things you can do for your long-term health and to reduce your risk of other chronic diseases.
- **Emotional Well-being:** Transitioning to survivorship can be challenging. Fear of recurrence, anxiety, and adjusting to a "new normal" are common. Please utilize our psychosocial oncology resources or seek professional support if you find these feelings are impacting your quality of life.

Your outcome to date has been outstanding, a testament to your resilience and the effectiveness of modern therapy for favorable-risk AML. We look forward to continuing to partner with you in your long-term health journey.